Hope Hospital		
		Guidance on Medications adminstration
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Purpose

To ensure safe, accurate, and standardized administration of medications to patients, thereby minimizing the risk of errors and ensuring compliance with NABH and legal standards.

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2. Scope

Applies to all nursing staff and authorized healthcare professionals involved in the administration of medications in inpatient and outpatient settings.

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3. Core Principles of Safe Medication Administration

- **Right Patient**
- **Right Medication**
- **Right Dose**
- **Right Route**

- **Right Time**
- **Right Documentation**
- **Right Reason**
- **Right Response**

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4. Authorized Personnel

- Only **qualified and trained healthcare professionals** (typically registered nurses) are permitted to administer medications.
- Administration of high-risk or injectable drugs should follow hospital-specific policies and require double-checking by two personnel where applicable.

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5. Pre-Administration Checks

- Verify the **prescription** (must be complete, legible, and signed by an authorized prescriber).
- Confirm:
  - Patient identity using **two identifiers** (e.g., name and UHID/ID band).
  - Allergy history (ensure "no known drug allergy" or list of allergies is documented).
  - Expiry date and integrity of the drug.
  - Drug interactions if the patient is on multiple medications.

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6. Medication Preparation

- Prepare medications in a **clean, well-lit, and quiet** area.
- Do not prepare medications in advance unless policy permits.
- Label medications if not administered immediately.
- Never administer medications prepared by others without rechecking.

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7. Routes of Administration

Follow standard procedures for different routes:

- **Oral**: Check patient's ability to swallow. Administer with water unless contraindicated.
- **IV/IM/Subcutaneous**: Ensure correct site, technique, and sterile equipment.
- **Topical/Inhalational/Ophthalmic/Otic**: Follow manufacturer instructions strictly.

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8. Administration Timing

- Administer at the time prescribed ±30 minutes for scheduled drugs (or as per hospital policy).
- **STAT or PRN orders** must be documented with administration time and reason.

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9. Documentation

- Record immediately after administration:
  - Time, dose, route, and initials of the person administering.
  - Any side effects or adverse reactions.
- Use electronic or manual medication administration record (MAR) system.

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10. Missed or Refused Doses

- Document any **missed or refused dose** clearly.
- Inform the treating doctor and pharmacist as appropriate.
- Record reason and any patient education provided.

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11. Adverse Drug Reaction (ADR) Management

- Monitor for side effects post-administration.
- Report any ADRs as per the hospital's ADR reporting protocol.

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12. High-Risk Medications

- Follow special precautions for high-alert drugs (e.g., insulin, chemotherapy, anticoagulants).
- Require **double verification** for high-risk medications.
- Store separately and clearly labeled.

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13. Infection Control Measures

- Hand hygiene before and after drug administration.
- Use gloves and appropriate PPE as required.

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14. Patient Education

- Educate patients about:
  - Purpose of the drug
  - How to take it (if self-administered)
  - Side effects to watch for
  - Compliance importance